Los Angeles County, California

COUNTY DATA BRIEF | Published response: Prioritize targeted follow-up | Score 85.6 | FIPS 06037

Why it surfaced The Atlas displays a published response of Prioritize targeted follow-up for Los Angeles County, California. The published gap score is 85.6. Based on published Atlas gap-score category. This is not a model-assisted surveillance priority. The notes below describe the published county record. They do not show how much each field contributed to the gap score, and they are not a measure of individual risk.

County data / Six review factors

01 **Published Lyme case data**
The published 2023 county case-count floor is 15. The minimum rate supported by that floor was 0.1 cases per 100,000. That floor is not a complete case count or incidence estimate. 43 published 2023 human-surveillance records were not allocated to a county in this release. Check whether any belong in this county before interpreting the county figure.

02 **Tick presence**
Published tick surveillance classifies this county as established. *Ixodes pacificus* is established. A published status is not a measure of tick abundance.

03 ***Borrelia burgdorferi* detection**
Borrelia burgdorferi was detected in tested ticks in the published surveillance data. Detection is not a human infection or transmission estimate.

04 **Social vulnerability**
The 2022 social vulnerability percentile is 86.3. Higher values indicate greater vulnerability on the source index. This is context for access and outreach, not proof of underreporting.

05 **Health insurance coverage**
The published percentage uninsured is 9.0%. This is context for healthcare access, not proof that cases are being missed.

06 **Rurality**
USDA Rural-Urban Continuum Code 1 (2023): metro area of 1 million or more. Code 1 is the most metropolitan and 9 is the most rural. This is context for follow-up planning, not a tick-habitat or underreporting measure.

Interpreting the data Lyme cases are assigned by county of residence, not necessarily the location of exposure. These data describe published surveillance, not individual risk. These values describe the published record. They do not show how much each field changed this county's gap score. The percentage uninsured is context; the score uses the uninsured percentile.

Observation periods: Lyme case records 2023; social vulnerability 2022; rurality 2023.

Prioritize targeted follow-up

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Recommended posture Consider giving greater attention to focused, locally governed review when the published response category is strongest.

Five suggested follow-up workstreams

Human disease surveillance	Consider prioritizing focused review of locally available Lyme disease surveillance information. Reconcile consequential published limitations, relevant case and exposure information, and applicable reporting practices to determine whether further public-health action is warranted. Compare the published county case-count floor with state or local surveillance records, including reporting completeness and residence-versus-exposure differences where investigation notes already exist. 43 published 2023 human-surveillance records were not allocated to a county in this release. Check whether any belong in this county before interpreting the county figure.
Vector and pathogen surveillance	Consider prioritizing review with vector surveillance partners of relevant tick and pathogen findings, including species, collection coverage, timing, and methods. Determine whether existing data are sufficient or whether narrowly targeted surveillance would inform a concrete decision. Review the published tick presence and *Borrelia burgdorferi* detection records, including the time and geographic coverage of collections where local records include that detail. Detection is not a human transmission estimate.
Clinical and laboratory reporting	Where local findings indicate a meaningful case-ascertainment or reporting concern, consider focused discussion with relevant surveillance, clinical, or laboratory-reporting partners. Any education or workflow review should respond to an identified need, not an assumption of reporting failure. Check whether clinical and laboratory reporting pathways can be compared with the published human and pathogen records. This county evidence does not indicate a test for any individual patient.
Community context and outreach	Consider whether locally verified evidence supports focused prevention education or outreach in particular communities or settings. Use local knowledge of communication needs and healthcare access; the Atlas response category alone does not justify a public warning. Social vulnerability, health insurance coverage, and rural–urban classification are available as context. Consider whether existing prevention education reaches the communities those measures describe before adding outreach. They are not evidence of transmission or underreporting.
Coordination and documentation	Consider prioritizing a defined, coordinated follow-up question using existing jurisdictional decision processes. Document supporting evidence, remaining uncertainties, possible actions, and local criteria for proceeding or stepping back. Document the outstanding evidence questions and identify appropriate surveillance or vector-program partners for local verification. Atlas does not assign that work or a completion date.

Questions before acting

01 What verified evidence makes this county a stronger candidate for focused follow-up than other current program priorities?
02 Have alternative explanations—including travel-associated exposure, surveillance limitations, and reporting practices—been considered?
03 What specific action could reduce important uncertainty or improve a locally supported public-health response?
04 What additional evidence, operational capacity, and local approval would be needed before proceeding?

Official resources & traceability

Lyme surveillance case definition | County case data and residence-versus-exposure limitations | Tick and pathogen record definitions and limitations | Clinical testing guidance | CDC Lyme disease prevention | Releases and methodology | Western blacklegged tick surveillance guidance | SVI interpretation | MRLC Annual NLCD 2025 reviewed demo cohort | USDA ERS Rural-Urban Continuum Codes | CDC/ATSDR Social Vulnerability Index | CDC Lyme surveillance

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Published gap score 85.6
Published response: Prioritize targeted follow-up
Data release governed-2026-10-15-january-climate-prod
Data methodology semantic-2.0.0
Score methodology surveillance-gap-score-v1.1; smooth k=50
Template county-v3 · copy explorer-follow-up-web-e6c0aa2
Observation periods: Lyme case records 2023; social vulnerability 2022; rurality 2023.
Source vintages: MRLC Annual NLCD 2025 reviewed demo cohort (C1V2-2025); USDA ERS Rural-Urban Continuum Codes (2023); CDC/ATSDR Social Vulnerability Index (2022 (2018-2022 ACS)); CDC Lyme surveillance (2023); CDC ArboNET tickborne pathogen status (through 2025-12-31); CDC ArboNET county tick status (through 2025-12-31)
Generated 2026-10-20 13:55 UTC